Prolonged Grief Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Prolonged grief disorder (PGD) is grief that stays intense, consuming, and disabling long after a death, well beyond what the person's culture and faith community would expect. It is **new in DSM-5-TR** (2022), placed with PTSD in "Trauma- and Stressor-Related Disorders" (American Psychiatric Association [APA], 2022).

How common: across 14 studies, about **1 in 10 bereaved adults** (9.8%) developed PGD, more often at older ages (Lundorff et al., 2017).

Most people are resilient. Following 205 people from before a spouse's death to 18 months after, resilience was the most frequent pattern; chronic grief was linked to dependency on the spouse before the loss (Bonanno et al., 2002).

Counselor framing: grief is not a disorder. PGD describes the minority whose grief becomes stuck; they respond well to targeted treatment and should not be told simply to "give it time."

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Timing:** the death of someone close at least **12 months** ago (6 months for children and adolescents).
- **Core grief response,** nearly every day for at least the last month: intense **yearning or longing** for the person, and/or **preoccupation** with thoughts or memories of them.
- **Three or more of:** feeling part of oneself has died (identity disruption); marked disbelief about the death; avoiding reminders that the person is dead; intense emotional pain (anger, bitterness, sorrow); difficulty re-engaging with relationships, activities, or plans; emotional numbness; feeling life is meaningless; intense loneliness.
- **Severity and context:** significant distress or impairment, and a duration and intensity that **clearly exceed the person's cultural, religious, and social norms**, not better explained by another disorder.

Distinct from depression and PTSD: in three samples, PGD overlapped only modestly with PTSD, major depression, and generalized anxiety, and the diagnosis was very stable over time (r = 0.86). The 13-item PG-13-Revised matched the DSM criteria well (κ = 0.70 to 0.89) (Prigerson et al., 2021).

Suicide risk: among 309 bereaved adults, complicated grief raised the odds of high suicidality 6.6-fold at about 6 months and 11.3-fold at about 11 months, even after accounting for depression and PTSD (Latham & Prigerson, 2004). Ask directly, including about wishes to "join" the deceased.

When counselors must refer (general clinical consensus): refer for psychiatric evaluation when there is suicidality, severe depression, or substance misuse. Violent deaths (suicide, homicide, accident) raise risk and call for trauma-informed care.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in prolonged grief. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PG-13-R (Prolonged Grief-13-Revised)	Screening for DSM-5-TR prolonged grief disorder.	Self-report, 13 items	No (free)	None beyond general clinical training	Original article
ICG (Inventory of Complicated Grief)	Severity of maladaptive grief symptoms; widely used in research.	Self-report, 19 items	No (free)	None beyond general clinical training	Original article

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PHQ-9 (Patient Health Questionnaire-9)	Screening and tracking depression; item 9 screens for suicidal thoughts.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
PCL-5 (PTSD Checklist for DSM-5)	Checking for PTSD after a traumatic or violent death.	Self-report, 20 items, 5 to 10 minutes	No (free)	None beyond general clinical training	National Center for PTSD
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening and assessing suicide risk; brief screener and full versions.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: grief-focused CBT with an exposure component works for PGD. Routine "preventive" grief counseling for everyone does not. Antidepressants ease depression but not grief itself.

Evidence overview (honest effect sizes):

- **Treatment, not prevention:** across 14 trials, treatment for people with complicated grief worked, while preventive interventions for all bereaved people did not (Wittouck et al., 2011). Across 31 trials, the pooled effect on grief was moderate (g = 0.41 after treatment, 0.45 at follow-up), larger for individual therapy and for people at least 6 months past the loss (Johannsen et al., 2019).
- **Complicated grief treatment** (a CBT-informed approach with revisiting the death, imaginal conversations, and restoring life goals): response 51% versus 28% with interpersonal therapy (number needed to treat 4.3) (Shear et al., 2005). In 395 adults, response was 82.5% with this treatment plus placebo pills versus 54.8% with placebo pills and supportive grief monitoring; adding citalopram did not improve grief but did reduce depression (Shear et al., 2016).
- **CBT with exposure:** in 54 mourners, both CBT sequences beat supportive counseling, and exposure added the most benefit (Boelen et al., 2007). In 80 patients, CBT plus exposure to memories of the death reduced PGD, depression, and negative appraisals more than CBT alone (Bryant et al., 2014).

CBT for grief: how it is used. Grief gets stuck through three processes: the loss is not integrated into memory, negative beliefs form about self, life, and the future, and the person avoids reminders or withdraws (Boelen et al., 2006). Healthy grieving moves back and forth between facing the loss and rebuilding life (Stroebe & Schut, 1999). Treatment therefore:

- **Identifies** stuck beliefs: "I can't live without her," "If I stop grieving I'm betraying him," "It's my fault," "Life has no meaning now" (Boelen et al., 2006).
- **Processes the death** through gentle, repeated retelling and by facing avoided places and reminders (Boelen et al., 2007; Bryant et al., 2014).
- **Challenges and replaces** catastrophic or guilt-laden beliefs, and **restores** valued activities, relationships, and goals (Shear et al., 2005).

5. Religion, spirituality, and Christian integration: what the evidence shows

Faith usually helps, but not always. A review of 73 studies found the links between religion and adjustment to bereavement generally positive but inconsistent, depending on how faith is measured (Wortmann & Park, 2008). In 32 studies (83% Protestant participants), 94% reported some benefit of religious or spiritual beliefs, but weak methods prevent firm conclusions (Becker et al., 2007).

Spiritual grief. Loss can also shake a person's relationship with God. Among 150 mourners, people bereaved by violent death reported more complicated grief and more "complicated spiritual grief," a crisis of faith after loss (Burke & Neimeyer, 2014). Among 1,056 bereaved students, difficulty making sense of the loss explained why violent deaths led to more complicated grief (Currier et al., 2006).

Intervention studies. We found **no randomized trial of Christian or religiously integrated CBT for prolonged grief disorder** (PubMed search, October 31, 2026). The evidence for faith comes from observational studies, not treatment trials.

Honest summary for teaching:

1. Grief-focused CBT with exposure, and complicated grief treatment, are the evidence base for PGD. Faith integration supports them; it does not replace them.
2. Faith is usually a resource in grief, but spiritual struggle and failure to make sense of the loss predict worse grief, especially after violent deaths. Assess both.
3. No faith-integrated PGD trial exists yet. Integrate Christian meaning-making carefully, with consent, inside an evidence-based approach.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within grief-focused CBT. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Ask how the loss has affected faith. Listen for spiritual stuck points: "God took her to punish me," "If I had prayed harder he would have lived," "Moving on means I didn't love him," or "A strong Christian wouldn't still be grieving" (Burke & Neimeyer, 2014). DSM-5-TR itself asks whether grief exceeds the person's religious norms, so understand those norms (APA, 2022).
- **Challenge (clinical).** Test guilt and meaning-making beliefs against the evidence and help the person make sense of the loss (Currier et al., 2006). Correct the belief that grieving shows weak faith.
- **Replace (pastoral).** Offer Scripture that honors grief and gives hope:
  - "Jesus wept." (John 11:35). Grief is not a lack of faith; Jesus grieved at His friend's tomb.
  - "…so that you will not grieve like the rest, who are without hope." (1 Thessalonians 4:13). Christians grieve, but with hope.
  - "Blessed are those who mourn, for they will be comforted." (Matthew 5:4)
  - "A time to weep and a time to laugh, a time to mourn and a time to dance." (Ecclesiastes 3:4). Re-engaging with life is not betrayal.
- **Faith community (clinical).** Encourage connection with church and family, and help the person return gradually to worship and activities they have avoided since the death (Boelen et al., 2006).
- **Ministers:** normal grief needs presence, not therapy. Refer when, after a year, grief remains intense and disabling, or at any time when there is suicidal thinking (Latham & Prigerson, 2004; Wittouck et al., 2011).

7. Quick classroom takeaways

- PGD requires 12 months since the death, daily yearning or preoccupation, 3 or more other symptoms, and grief clearly beyond cultural and religious norms. Screen with the PG-13-R and always ask about suicide.
- Grief-focused CBT with exposure and complicated grief treatment work; preventive counseling for everyone does not; antidepressants help depression, not grief.
- Faith usually helps, but spiritual grief is real. Challenge beliefs that grieving is faithless or that moving on is betrayal, and replace them with the hope of the resurrection.
- Screen with the free PG-13-R, and check for depression, PTSD, and suicide risk alongside grief.

Sources note

All 17 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Becker, G., Xander, C. J., Blum, H. E., Lutterbach, J., Momm, F., Gysels, M., & Higginson, I. J. (2007). Do religious or spiritual beliefs influence bereavement? A systematic review. *Palliative Medicine*, *21*(3), 207–217. https://doi.org/10.1177/0269216307077327

Boelen, P. A., van den Hout, M. A., & van den Bout, J. (2006). A cognitive-behavioral conceptualization of complicated grief. *Clinical Psychology: Science and Practice*, *13*(2), 109–128. https://doi.org/10.1111/j.1468-2850.2006.00013.x

Boelen, P. A., de Keijser, J., van den Hout, M. A., & van den Bout, J. (2007). Treatment of complicated grief: A comparison between cognitive-behavioral therapy and supportive counseling. *Journal of Consulting and Clinical Psychology*, *75*(2), 277–284. https://doi.org/10.1037/0022-006X.75.2.277

Bonanno, G. A., Wortman, C. B., Lehman, D. R., Tweed, R. G., Haring, M., Sonnega, J., Carr, D., & Nesse, R. M. (2002). Resilience to loss and chronic grief: A prospective study from preloss to 18-months postloss. *Journal of Personality and Social Psychology*, *83*(5), 1150–1164. https://doi.org/10.1037/0022-3514.83.5.1150

Bryant, R. A., Kenny, L., Joscelyne, A., Rawson, N., Maccallum, F., Cahill, C., Hopwood, S., Aderka, I., & Nickerson, A. (2014). Treating prolonged grief disorder: A randomized clinical trial. *JAMA Psychiatry*, *71*(12), 1332–1339. https://doi.org/10.1001/jamapsychiatry.2014.1600

Burke, L. A., & Neimeyer, R. A. (2014). Complicated spiritual grief I: Relation to complicated grief symptomatology following violent death bereavement. *Death Studies*, *38*(4), 259–267. https://doi.org/10.1080/07481187.2013.829372

Currier, J. M., Holland, J. M., & Neimeyer, R. A. (2006). Sense-making, grief, and the experience of violent loss: Toward a mediational model. *Death Studies*, *30*(5), 403–428. https://doi.org/10.1080/07481180600614351

Johannsen, M., Damholdt, M. F., Zachariae, R., Lundorff, M., Farver-Vestergaard, I., & O'Connor, M. (2019). Psychological interventions for grief in adults: A systematic review and meta-analysis of randomized controlled trials. *Journal of Affective Disorders*, *253*, 69–86. https://doi.org/10.1016/j.jad.2019.04.065

Latham, A. E., & Prigerson, H. G. (2004). Suicidality and bereavement: Complicated grief as psychiatric disorder presenting greatest risk for suicidality. *Suicide and Life-Threatening Behavior*, *34*(4), 350–362. https://doi.org/10.1521/suli.34.4.350.53737

Lundorff, M., Holmgren, H., Zachariae, R., Farver-Vestergaard, I., & O'Connor, M. (2017). Prevalence of prolonged grief disorder in adult bereavement: A systematic review and meta-analysis. *Journal of Affective Disorders*, *212*, 138–149. https://doi.org/10.1016/j.jad.2017.01.030

Prigerson, H. G., Boelen, P. A., Xu, J., Smith, K. V., & Maciejewski, P. K. (2021). Validation of the new DSM-5-TR criteria for prolonged grief disorder and the PG-13-Revised (PG-13-R) scale. *World Psychiatry*, *20*(1), 96–106. https://doi.org/10.1002/wps.20823

Shear, K., Frank, E., Houck, P. R., & Reynolds, C. F. (2005). Treatment of complicated grief: A randomized controlled trial. *JAMA*, *293*(21), 2601–2608. https://doi.org/10.1001/jama.293.21.2601

Shear, M. K., Reynolds, C. F., Simon, N. M., Zisook, S., Wang, Y., Mauro, C., Duan, N., Lebowitz, B., & Skritskaya, N. (2016). Optimizing treatment of complicated grief: A randomized clinical trial. *JAMA Psychiatry*, *73*(7), 685–694. https://doi.org/10.1001/jamapsychiatry.2016.0892

Stroebe, M., & Schut, H. (1999). The dual process model of coping with bereavement: Rationale and description. *Death Studies*, *23*(3), 197–224. https://doi.org/10.1080/074811899201046

Wittouck, C., Van Autreve, S., De Jaegere, E., Portzky, G., & van Heeringen, K. (2011). The prevention and treatment of complicated grief: A meta-analysis. *Clinical Psychology Review*, *31*(1), 69–78. https://doi.org/10.1016/j.cpr.2010.09.005

Wortmann, J. H., & Park, C. L. (2008). Religion and spirituality in adjustment following bereavement: An integrative review. *Death Studies*, *32*(8), 703–736. https://doi.org/10.1080/07481180802289507

Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 17 references and DOIs verified via Crossref and PubMed (Saturday, Oct 31, 2026). Effect sizes as reported in the cited sources.